



P07 · 40 PAGES, FIRST SIX WEEKS

## *The Newborn Survival Planner*

*Forty pages of survival-mode scaffolding for the first six weeks home with a newborn. Day-by-day expectations, a feeding and diaper log that doesn't shame you for forgetting, a visitor scripts page, and a "what's normal" reference for the questions you'd otherwise google at 3am.*

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## *Soothemade* Notes

A SMALL APOTHECARY OF PRINTABLES · MADE SLOWLY

## A note before you start

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I am not a pediatrician. I am not a postpartum doula. I am a parent who, with my first, brought home a baby and stood in the kitchen at 3am crying because I could not remember whether I had fed him at 1am or 11pm.

The hospital sent us home with a folder of glossy pamphlets, none of which addressed the actual question I had, which was "is this normal." A nurse handed me a piece of paper that said "Sleep when the baby sleeps." I'd already been awake for thirty hours. I wanted to set the paper on fire.

This planner is the thing I wish I'd had on the kitchen counter. Not a methodology. Not a parenting philosophy. A scaffolding for the kind of week where you cannot keep track of anything.

It assumes you are doing your best. It assumes some days will be worse than the day before. It assumes you will sometimes skip a page. All of that is allowed.

With care, Maya

*None of this is medical advice. If something feels wrong with your baby or with you, call your provider. The trusted numbers are listed in the back of this planner. If you are in crisis, do not look it up in here. Call 988 or the US Maternal Mental Health Hotline at 1-833-852-6262.*

## *The one thing to remember*

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**I**f you remember nothing else from this planner, remember this:

**The first six weeks are not a smaller version of parenting later. They are a separate event.**

It is not “the start of motherhood, but harder.” It is its own time. Survival mode is the correct mode. The “real you” is not coming back this week. The version of you who is here now is enough.

Six weeks is also not a magic number. You will not wake up on day forty-three and be sleeping through the night. But something does shift, somewhere in there, almost universally. Hold on to that.

# The first 48 hours

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The first two days, hospital and homecoming, run on their own clock. Here is what to actually do.

## *Do these things, no asking*

- Drink water every time the baby feeds. Set a glass in your reach. Refill it without ceremony.
- Take photos. The first feed. The first diaper. You holding the baby. The partner holding the baby. You do not have to post any of them. They are for you.
- Write down what the nurses tell you. Discharge instructions get fuzzy fast. There is a page in this planner for that.
- If you can, do not put yourself in charge of any household decision yet. Not the laundry. Not the meal plan. Not the thank-you cards.
- Sleep when you can. Not when the baby sleeps. When you can. Sometimes that is at 4pm.

## *What you will be tempted to do (don't)*

- × Send a long birth-story text to everyone you've ever met. You can do that in week three. Send a single photo and the baby's name. That is enough.
- × Make a list of thank-yous. The list will keep. The list will still be there in eight weeks.
- × Try to shower in a way that feels normal. You are not normal. The shower can be three minutes. The shower can be a wet washcloth. Both count.
- × Google "is my baby breathing normally" at 2am. Watch the baby. If you are unsure, call the nurse line. Do not crowdsource.

### *A note on tears*

If you cry on day three, that is the baby blues. About 80% of birthing parents have a wave of intense emotion between day three and day five. It does not mean you are not bonding. It does not mean you have postpartum depression yet. It means your hormones are doing a forty-foot drop in seventy-two hours.

If the tears are still daily by week three, that is a thing to flag. There is a page on this in the back.

# *Week 1: survival mode*

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**W**eek one is the brownout. You are not making complex decisions. You are keeping a human alive with your body, while also healing from something the size of major surgery.

## *The priorities, in order*

### **1. Feeding the baby**

Every two to three hours, around the clock. The baby will cue. You will learn the cues. In the meantime, if it has been longer than three hours, wake them.

If breastfeeding, watch for: ten to twelve feeds a day, audible swallows, six to eight wet diapers by day five, soft-yellow stools by day five.

If formula-feeding, that's about 1.5 to 2 ounces per feeding by end of week one, every three hours-ish.

If something feels wrong with feeding, call. There is a lactation page in the back and a number for the US National Breastfeeding Helpline.

### **2. Feeding yourself**

Every time you feed the baby, eat or drink something. A handful of nuts. Half a piece of toast. A glass of water. If you are nursing, double that.

Keep snacks within arm's reach of every place you sit or lie down. The bedside. The nursing chair. The couch. Build a snack drawer once and stop thinking about it.

### **3. Sleep, in any form**

You are not getting eight hours. You may not be getting four in a row. The goal of week one is not "well-rested." The goal is "did not get below the line."

Take any nap you can. Take a nap at 10am if it presents itself. Sleep is sleep.

#### 4. The bleeding situation

If you gave birth, you are bleeding. It is heavy for about a week, then tapers. It can last up to six weeks total.

You are watching for: a pad soaked through in less than an hour, clots bigger than a golf ball, a foul smell, a fever above 100.4°F. Any of those, call your provider. Today.

#### 5. Visitors

Do not have any yet. Or have only two. With food. For two hours. No exceptions.

There is a visitor scripts page in this planner. Hand it to your partner.

### *The daily check-in*

A template you will see thirty times in the printed planner:

DAY \_\_\_\_ / DATE \_\_\_\_\_ / BABY'S AGE: \_\_\_\_ days

FEEDS LAST 24H: ☐ ☐ ☐ ☐ ☐ ☐ ☐ ☐ ☐ ☐ ☐ ☐ ☐ ☐ ☐

WET DIAPERS: ☐ ☐ ☐ ☐ ☐ ☐ ☐ ☐

DIRTY DIAPERS: ☐ ☐ ☐ ☐ ☐

BABY:

Color of skin (circle): pink pale yellow-tinted

Alertness when awake: bright normal sleepy

One thing I noticed today: \_\_\_\_\_

ME (birthing parent):

Pad changes today: \_\_\_\_ Pain (0-10): \_\_\_\_

Did I eat? (circle): yes barely no

Did I drink water? yes barely no

Mood, in one word: \_\_\_\_\_

US:

Best moment today: \_\_\_\_\_

Hardest moment today: \_\_\_\_\_

One question to ask the pediatrician: \_\_\_\_\_

Do it with one hand, in any handwriting. Skip lines. Cross out. This is not a museum object.

## *Week 2: the brownout deepens*

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**W**eek two is when the sleep debt becomes a fact, not a hypothesis.

The visitors who came in week one are gone. The freezer meals are running low. The partner may be heading back to work, or trying to.

This is the week where the kitchen counter slowly fills up with mail you have not opened. That is allowed. The mail can wait.

### ***What's new this week***

- The baby is awake for slightly longer stretches. Not happy stretches yet. Just awake.
- You may notice you have stopped feeling like a person. That happens. It is not a sign of anything besides week two.
- Cluster feeds get more intense. The baby may feed every forty-five minutes from 5pm to 10pm. That is a growth phase, not a supply problem.
- The umbilical cord stump usually falls off this week. There may be a little blood on the diaper when it does. That is normal. If there is pus, or it smells, call.

### ***What to let go***

- Replying to texts. Set a single auto-reply if you can. "Thank you for thinking of us. We are doing well. I will be quiet for a few weeks."
- The laundry pile. There is no laundry score. The clothes go from clean to dirty to clean again. The pile is not a referendum on you.
- The aspirational version of week two you saw on instagram. That person had a postpartum doula, a night nurse, and a photographer.

## *What to add, if you have one good hour*

If you have one good hour this week (you might not, that's fine), use it for one of these:

- Schedule the two-week pediatrician visit if not already done.
- Make a list of three meals that are not freezer meals. Send the list to whoever asked "is there anything I can do?" with a delivery address.
- Take a fifteen-minute walk to the corner and back. Outside light helps your sleep, even when sleep is broken.

## *Week 3: the witching hour*

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**W**eek three is famously the worst. Sorry.

This is the week where evenings turn into a sustained, inconsolable cry from the baby, usually starting around 5pm and lasting one to three hours. It is called the witching hour even though it is rarely just an hour.

It is not because you are doing something wrong. It is not because of dairy in your diet. It is the baby's nervous system trying to process a full day's worth of input with no off-switch.

### *The witching hour toolkit*

Try, in this order:

1. **The five S's, in sequence.** Swaddle. Side-or-stomach hold (in your arms, not for sleep). Shush, loud, right at the ear. Swing, small and rhythmic. Suck, pacifier or your knuckle.
2. **Take the baby outside.** Even if it is dark. Even five minutes. Air does things.
3. **A warm bath with you in it.** If you are healed enough, get in the tub with the baby. The water often resets both of you.
4. **Hand off.** If there is another adult, take a twenty-minute break and let them try. Different chest, different motion.
5. **Accept it.** Sometimes there is no fix. The baby cries. You hold the baby. The crying ends, eventually. You both survive.

You are also allowed, mid-witching-hour, to put the safely-supervised baby down in the crib and walk outside for two minutes to breathe. The baby will not be harmed. You are not a worse parent for it.

## *What if I think something is actually wrong*

The witching hour is normal. Concerning crying is different. Concerning crying:

- Is high-pitched, sustained, and unlike the baby's usual cry
- Is paired with a fever (over 100.4°F rectally under 3 months)
- Is paired with the baby pulling legs to chest and going pale
- Lasts more than three hours and the baby is also refusing feeds or floppy

Any of those, call. Today. Not tomorrow.

## Week 4: the half-smile

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**W**eek four is when something shifts.

It might be a real eye-contact moment that lasts more than half a second. It might be a half-smile, the kind that is not gas. It might be a stretch of forty-five minutes where the baby is awake, alert, and not crying.

It is small. You will not put it on social media because it does not photograph. But you will know.

This is also the week where the baby starts to learn your face is a thing they can look at on purpose.

### *What to track this week*

- First social smile (yes, write down the date when it happens)
- First sustained eye contact
- The first time you laughed at something the baby did

### *What to expect with sleep*

Sleep is still broken. But the longest stretch might start to extend, slowly. Three hours becomes three and a half. Three and a half becomes four. Not every night. Some nights are still week one. The trend is what matters, not any single night.

### *The four-week pediatrician visit*

If you have not already, schedule it. There will be a weight check, a head measurement, and questions about how feeding is going. There is an appointment prep page later in this planner, two pages back from the colophon.

Things to write down before you go:

- Your one biggest question about the baby
- Your one biggest question about yourself
- The diaper output range for the last three days
- The longest stretch of sleep the baby is doing
- Any rash, spit-up pattern, or feeding worry

The visit is short. Bring the questions in writing or they fly out of your head.

## *Week 5: the first stretch*

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**W**eek five is the week the longest sleep stretch might, possibly, on one night, become five hours.

You will not believe it when it happens. You will wake up at 3am sure that you have killed the baby through inattention. The baby will be fine. You will lie in the dark trying to decide whether to check on them or sleep. (Check on them, then sleep.)

### ***What is normal this week***

- A longer wakeful window in the morning, 60 to 90 minutes
- More predictable feeds, every 2.5 to 3 hours rather than every 1.5
- A clearer "tired cry" vs "hungry cry" vs "wet cry," if you are listening for it
- The witching hour starts to shorten

### ***What is also normal***

- Days where none of the above is true and you are back in week one
- Tears, on your part, with no clear trigger
- The thought "I am not going to make it" several times a day. You are going to make it. Saying that thought out loud often helps it shrink.

### ***A small invitation***

In the printed planner, there is a one-page invitation at week five that says: pick one fifteen-minute thing this week that is only for you. Not "for the family." Not "self-care for moms." A fifteen-minute thing that the version of you before this would have wanted.

A coffee that is not in a travel mug. Five pages of a book. Putting on real shoes. Listening to one full song.

If you cannot do it this week, that's fine. The invitation will be there in week seven also.

## *Week 6: the reckoning*

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**W**eek six is the conventional finish line. The six-week postpartum visit. The “back to normal activities” green light.

You will not feel back to normal. That is fine. The green light is medical, not emotional. Your pelvic floor may or may not be ready for what your provider says it’s ready for. Your hormones will keep shifting. Your sense of who you are is not going to snap back this week.

### ***What the six-week visit actually checks***

- Healing of perineum and any stitches
- Healing of a C-section incision, if applicable
- Uterus has returned to pre-pregnancy size
- Blood pressure
- Mood (they will ask, with a brief screening)
- Birth control plans, whether or not you want to discuss them
- Sex, whether or not you want to discuss it

You are allowed to say “I am not ready” to any of these.

### ***What to bring up if they don’t***

- Bladder leaks
- Pain during anything
- Diastasis (the gap in your abdominals)
- Persistent low mood, or mood that swings in a way that doesn’t feel like just-tired
- Any feeding question that is still unresolved
- Any concern about the baby that the pediatrician didn’t address

## *The end of six weeks page*

In the printed planner, the last week-page is followed by a single page with this on it:

*The version of you reading this page is not the same person who started this planner six weeks ago. You don't have to be impressed with yourself. You don't have to celebrate. You don't have to take a photo of the page.*

*You kept a person alive for six weeks. The person is breathing. You are breathing.*

*That is the thing.*

## The visitor scripts page

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A separate page in the planner. The partner can hand it out. You can text it. You can stick it on the door.

Thank you for wanting to meet the baby.

We are not having visitors until week \_\_\_\_.

After that, we are having two people at a time, for two hours,  
who are bringing food.

We will not be hosting. We will not be making coffee.  
You may have to let yourself in.

We love you. We will see you when we can.

— \_\_\_\_\_ and \_\_\_\_\_

A second version, for whoever keeps asking when they can come:

We are doing okay. Not great, but okay. The week three crying  
phase is real and we are tired in a way that does not photograph.

We are not ready for visitors yet. We will reach out when we are.  
Please do not take the silence personally.

If you would like to help: a meal left on the porch is the  
single most useful thing right now.

— \_\_\_\_\_ and \_\_\_\_\_

# *The 3am google: what's normal*

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A reference for the questions you would otherwise type into a search bar with one hand at 3am.

## ***Cluster feeding***

Feeding every 30-45 minutes for several hours, usually evening. Common at week 2-3, week 4-6, and any growth spurt. Not a supply problem. Not a "fussy baby." Normal.

## ***Witching hour***

Sustained crying, usually 5pm to 8pm-ish, peaks around week 6. Normal. The crying ends. Not your fault. Try the five S's. Pass the baby to someone else if you can.

## ***Hiccups***

Newborn hiccups can last 10-20 minutes and look distressing. They almost always resolve on their own. The baby is fine. They don't hurt.

## ***Spit-up***

A small amount after every feed, even multiple times, is normal as long as the baby is gaining and not refusing feeds. Forceful projectile vomiting that flies a foot or more is different. Call.

## ***Cradle cap***

Yellowish flaky scalp. Cosmetic. Will resolve. Don't pick at it. A bit of coconut oil and a soft brush before bath.

## ***Baby acne***

Real. Appears around week 2-3. Resolves around week 6. Do not put face cream on a newborn.

## ***Cord stump***

Falls off any time between days 5 and 21. A spot of blood on the diaper the day it falls off is normal. Pus, redness extending into the belly skin, or smell is not. Call.

## ***Diaper output***

By day 5: at least 6 wet, 3-4 dirty. Stools transition from black-tarry to greenish to soft mustard-yellow by day 5-7. Mostly seedy and loose, not formed. Red, white-clay, or black-after-day-5 is a call.

## ***Fever***

A rectal temperature over 100.4°F (38°C) in a baby under 3 months is an ER visit. Not "wait and see." Not "give a fever reducer at home." A call, then often a trip.

## ***Cold***

Stuffy nose, mild fussiness, slight cough, normal feeds, no fever. Saline drops and a bulb syringe. Cool-mist humidifier. Comfort. Call the nurse line if it's been more than 2 days or if it gets worse.

## ***When to call no matter what***

- Fever as above
- Refusing two feeds in a row
- Vomiting that is forceful or green-tinted
- Lethargic, hard to wake, or floppy
- Trouble breathing, fast breathing, grunting, or nostrils flaring
- A rash that doesn't blanch when you press a glass on it
- Any wound or rash that looks infected

- Any time your gut says something is wrong

That last one is the most important. You know this baby. The phone is a tool. Use it.

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Pad changes today: \_\_\_\_\_

Breasts: ☐ comfortable ☐ engorged ☐ tender

Mood today: ☐ ok ☐ tearful ☐ overwhelmed ☐ flat

## The one thing I want

The one thing I'd

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Walking: ☐ normal ☐ sore ☐ painful

this week? ☐ yes ☐ no

## ***Weeks 3-4 check-in***

(same, plus:)

Have I held the baby  
without the baby?   ☐ yes   ☐ no  
Have I eaten a meal  
sitting down?   ☐ yes   ☐ no  
Have I cried more than  
twice this week?   ☐ yes   ☐ no   ☐ I'm not sure

## ***Weeks 5-6 check-in***

(same as above, plus:)

The signs I'd flag at  
my 6-week appointment: \_\_\_\_\_  
\_\_\_\_\_  
\_\_\_\_\_

# Pediatrician appointment prep

A template you'll see four times in the planner, one for each well-baby visit in the first six weeks (typically 3-5 days, 2 weeks, 1 month, and the 6-week if your provider runs that one).

APPOINTMENT FOR: \_\_\_\_\_

DATE: \_\_\_\_\_ TIME: \_\_\_\_\_

## THINGS THE PEDIATRICIAN WILL ASK ABOUT

- ☐ Feeding frequency and approximate intake
- ☐ Wet and dirty diaper count, last 24h
- ☐ Longest sleep stretch
- ☐ Any rashes, redness, swelling
- ☐ Any concerns you have

## MY QUESTIONS (write before you go, otherwise they fly out)

1. \_\_\_\_\_
2. \_\_\_\_\_
3. \_\_\_\_\_

## NOTES FROM THE VISIT

Weight: \_\_\_\_ lb \_\_\_\_ oz percentile: \_\_\_\_

Length: \_\_\_\_ in percentile: \_\_\_\_

Head circumference: \_\_\_\_ cm

Pediatrician said: \_\_\_\_\_

\_\_\_\_\_  
\_\_\_\_\_

NEXT APPOINTMENT: \_\_\_\_\_ at \_\_\_\_\_

## *The reset, week seven*

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A single page, the second-to-last in the printed planner.

*You made it to week seven. Or week eight. Or week twelve, because you bought this late, and that is allowed too.*

*The first six weeks were not a smaller version of what comes next. They were their own thing. You don't have to integrate them. You just have to put them down.*

*Here are three small questions for the version of you who is here now. Write the answers in the planner, or write them in your head, or skip them.*

*One. What is one thing I learned about myself in the last six weeks that I did not know before?*

*Two. What is one thing I want to ask for help with, in the next six weeks, that I would have tried to do alone?*

*Three. What is one thing I want to remember about the baby right now, before it changes?*

*That's it. Close the planner. Put it on a shelf. You can come back to it for the next baby, or for a friend's baby, or never.*

*Soothemade.*

# Numbers to keep

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A page at the very back of the planner. Fill it in once.

## PEDIATRICIAN

Name: \_\_\_\_\_ Number: \_\_\_\_\_

After-hours line: \_\_\_\_\_

## OB / MIDWIFE

Name: \_\_\_\_\_ Number: \_\_\_\_\_

After-hours line: \_\_\_\_\_

## LACTATION CONSULTANT

Name: \_\_\_\_\_ Number: \_\_\_\_\_

## POSTPARTUM DOULA (if any)

Name: \_\_\_\_\_ Number: \_\_\_\_\_

## US NATIONAL BREASTFEEDING HELPLINE

1-800-994-9662 (Mon-Fri, English + Spanish)

## US MATERNAL MENTAL HEALTH HOTLINE

1-833-852-6262 (24/7, free, confidential)

## US SUICIDE & CRISIS LIFELINE

988 (call or text, 24/7)

## POISON CONTROL (US)

1-800-222-1222 (24/7)

## LOCAL ER

Hospital: \_\_\_\_\_ Number: \_\_\_\_\_



*From Soothemade Notes, a small apothecary of printables, planners, and cards for the unphotographed parts of new parenthood. Made slowly, in plain language.*



# Soothemade *Notes*

*Made for the version of you no one tells you about, the  
one at four in the morning, the one in the parking lot, the  
one who doesn't recognize her own week.*

[notes.soothemade.com](https://notes.soothemade.com)

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